



# National Outcome and Case Mix Collection Procedure

## 1. Applicability

| Site                           | Operational Area | Applicable to                                         |
|--------------------------------|------------------|-------------------------------------------------------|
| Armadale Mental Health Service | All areas        | Medical<br>Nursing<br>Allied Health<br>Administration |

## 2. Purpose

The purpose of this procedure is to ensure consumer data is captured accurately at each episode of care.

## 3. Background

From 2022/23, the Australian Mental Health Care Classification (AMHCC) will be used as the base Activity Based Funding (ABF) model for mental health services. The AMHCC replaces the existing Diagnostic Related Group (DRG) funding model. This means that phase of care and outcome measures, such as the Health of the Nation Outcome Scales (HoNOS) are mandatory for completion for consumers per episode of care.

## 4. Access to Psychiatric On-line Information System (PSOLIS)

PSOLIS is designed to collect demographic information and treatment related history from consumers of public Mental Health Services (MHS) in order to support the provision of optimum care and treatment. All information is held in a secure manner and is kept private and confidential. It is only available to personnel authorised to access the information. Authorised persons are those involved in the treatment and care of clients of public MHS and have signed a document stating that they will abide by all privacy and confidentiality policies of the Department of Health.

Access to PSOLIS is restricted to staff members employed in a public MHS who have been issued with a HE number and granted access via the Line Management approval process.

The following exclusions should be observed:

- Staff with generic (shared) HE numbers will not be permitted access to PSOLIS.
- Agency staff and/or students will not be permitted access to PSOLIS. If required, a student's Supervising Officer's HE number is to be recorded on PSOLIS and the work can be jointly signed off.

In exceptional circumstances, students who are providing direct clinical services to consumers, independently, can be issued a HE number at the discretion of the Service Director, Mental Health.

All clinicians starting with Armadale Health Service and provided with access to PSOLIS will be provided with the document “Important Policy information for PSOLIS Users” by the PSOLIS Local Administrator.

Clinical staff are required to record data collected in compliance with the [National Outcome Case Mix Collection \(NOCC\) protocol](#) into PSOLIS. The accurate entry of all data collected after completing outcome measures is a critical success factor for the NOCC. Consumer self-reports such as, but not limited to, the Kessler 10 and Kessler 10+ will be placed in the medical record in hard copy. In addition, a hard copy of the summary graphs and/or summary reports generated by PSOLIS is to be stored in the appropriate section of the medical record or scanned into the Digital Medical Record.

## 5. Training requirements

Staff are mandated to undertake training in PSOLIS on a one-off basis at commencement of employment. PSOLIS training is to be booked with the Mental Health Information Systems (MHIS) Coordinator. NOCC training is available online via the Access My Learning on the EMHS Hub page. Refresher training for PSOLIS is available by arrangement with the MHIS Coordinator.

## 6. Measures specific to adults and older persons

Measures will be completed by clinicians in accordance with the requirement of the NOCC protocol identified above and at times identified by Appendix 2. Measures that will be collected for adult and older adult consumers include, but are not limited to:

- HoNOS/HoNOS65+ (Health of the Nation Outcome Scales):
- LSP-16 (Life Skills Profile)
- RUG-ADL (Resource Utilisation Groups – Activities of Daily Living Scale) - for over 65s only; and
- Kessler-10 (K-10) and Kessler -10+ (K-10+).

### 6.1 Health of the National Outcomes Scales (HoNOS and HoNOS 65+)

The HoNOS is a tool used by clinicians to measure consumer outcomes. It was designed specifically for use with people with a mental illness and is best considered as a general measure of severity of mental health disorder.

The HoNOS is not a formalised interview. Mental health staff gather information from all sources available to them to complete the measure including collateral information from key informants, review of the medical records, previous HoNOS ratings, as well as interview and observation. There are occasions when some information is not readily available; but be that as it is, mental health staff are still undertaking an assessment and making clinical decisions regarding the best approach to initial or ongoing management of the consumer based on that assessment. Given this clinical process, the HoNOS can still provide a summary of the clinician’s assessment based on the best available information. As a result, the reality is that the HoNOS can be completed during telephone triage and discharge processes, including lost to care and other occasions when the consumer is not present.

The focus of the HoNOS is on health status and severity of symptoms. It consists of 12 items that cover the sorts of problems that may be experienced by people with a significant mental illness with each item rated on a five-point scale. In assigning ratings, the clinician makes use of a glossary table which details the meaning of each point on the scale being rated. The clinician

rates the consumer on each of the scales in terms of their assessment of the person's situation over the recent period, usually defined as the previous two weeks. The exception is at discharge from inpatient services, based on the previous 3 days including the day of discharge.

The HoNOS 65+ is a variant of the general adult version of the HoNOS. The HoNOS 65+ consists of the same item set as the HoNOS and is scored in the same way; however, the accompanying glossary has been modified to better reflect the problems and symptoms likely to be encountered when assessing older persons.

***The use of 9 as a response to any question is no longer allowed.***

## **6.2 The Abbreviated Life Skills Profile (LSP-16)**

The Life Skills Profile, also known as LSP, was developed to assess a consumer's abilities with respect to basic life skills. Its focus is on the consumer's general functioning and disability rather than their clinical symptoms – that is, how the person functions in terms of social relationships, ability to do day-to-day tasks and so forth. When combined with the HoNOS, which requires ratings of the most serious problem encountered, the LSP contributes towards gaining a more comprehensive understanding of the consumer. The clinician is required to rate the consumer's overall situation over the past three months.

The LSP will generally be used with consumers seen in the community and for those undergoing extended-stay residential cares. This is because the LSP is largely designed to measure longer-term functioning and the three-month rating period is not suited to brief episodes of hospital care.

***The use of 7 or 8 as a response to any question is no longer allowed.***

## **6.3 Resource Utilisation Groups – Activities of Daily Living (RUG-ADL)**

This measure is only applicable to consumers aged 65 years and over. The RUG-ADL measures ability with respect to what are called 'late loss' activities – those activities that are likely to be lost in life (e.g. eating, mobility). 'Early loss' activities (such as dressing and grooming) are included in the LSP.

To complete the RUG-ADL, clinicians are asked to rate the consumer's needs for assistance in four activities of daily living: bed mobility, toileting, transfer and eating.

## **6.4 Kessler-10 (K10) and Kessler Plus (K10+)**

The K10 is a ten-item self-report questionnaire intended to yield a global measure of 'non-specific psychosocial distress' based on questions about the level of nervousness, agitation, psychological fatigue and depression in the relevant rating period.

The K10+ contains additional questions to assess functioning and related factors. Overall, the K10+ is a brief 'symptom and functioning' measure, validated against diagnosis, that is intended to be supplemented with additional measures of domains relevant to consumers.

The standard rating period is the 'last four weeks'.

## **6.5 Phase of Care**

The mental health phase of care is defined as the prospective primary goal of treatment within the episode of care in terms of the recognised phases of mental health care. Whilst it is recognised that there may be aspects of each mental health phase of care represented in the consumer's mental health plan, the mental health phase of care is intended to identify the main goal or aim that will underpin the next period of care. The mental health phase of care is

independent of both the treatment setting and the designation of the treating service and does not reflect service unit type.

- **Acute:** The primary goals of care are intended to reduce high levels of distress, manage complex symptoms, contain, and reduce immediate risk.
- **Functional Gain:** The primary goal of care is to improve personal, social, or occupational functioning or promote psychosocial adaptation in a patient with impairment arising from a psychiatric disorder.
- **Intensive Extended:** The primary goal of care is prevention or minimisation of further deterioration, and reduction of risk of harm in a patient who has a stable pattern of severe symptoms, frequent relapses, or severe inability to function independently and is judged to require care over an indefinite period.
- **Consolidating gain:** The primary goal of care is to maintain the level of functioning, or improving functioning during a period of recovery, minimise deterioration or prevent relapse where the patient has stabilised and functions relatively independently. Consolidating gain may also be known as maintenance.

NOTE: Previously reported Phase of Care - Assessment Only has been removed as a mental health phase of care and redefined as an administrative data item.

## 6.6 Key reference

Independent Hospital Pricing Authority (2016) Australian Mental Health Care Classification: Mental health phase of care guide. Ver 1.2 [Mental Health Phase of Care Guide | Resources | IHACPA](#)

## 6.7 Mandatory Collection Occasions for the NOCC

Mandatory collection occasions are noted in Appendix 2. Appendix 1 details common questions regarding the interpretation of the NOCC protocol in relation to particular circumstances. In all cases the minimum collection occasion is auto populated in PSOLIS, however extra measures may also be added.

## 7. Compliance monitoring

Compliance against this procedure will be monitored by the extraction of the Assessment Due report for each AHS Mental Health stream, from PSOLIS by the Mental Health Information Systems Coordinator. Compliance with NOCC entry requirements will be evaluated by extraction of data from the MIND database and reported to the Program Manager and Nurse Coordinator on a monthly basis.

## 8. Legislative context

The following documents are required to give effect to this procedure:

- *Mental Health Act 2014*
- HDWA [Admission Policy Reference Manual 2020-21](#)
- HDWA [Information Security Policy MP 0067/17](#)
- EMHS [Care Coordination in Mental Health](#)

- Armadale Health Service [Assessment Treatment Team Process Manual](#)

## 9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 6 – Action 6.11

## 10. References

1. Australian Mental Health Outcomes and Classification Network ([AMHOCN](#))
2. Independent Health and Aged Care Pricing Authority ([IHACPA](#))

## 11. Document contact

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Enquiries relating to this procedure may be directed to:

Department: Mental Health

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## 12. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

| Version | Effective from | Effective to | Amendment(s)                       |
|---------|----------------|--------------|------------------------------------|
| V2      | 27/10/2023     | 27/10/2026   | Scheduled review – minor amendment |

## 13. Authorisation

This procedure has been authorised and issued by:

|                    |                                                |
|--------------------|------------------------------------------------|
| Authorised by      | Karen Elliott – Service Director Mental Health |
| Authorisation date | 26/10/2023                                     |
| Published date     | 27/10/2023                                     |

**Appendix 1: Table 9.2 Recommended approach to special issues in interpreting the protocol at service delivery level of NOCC Protocol 2.10 (from amhocn.org)**

| Scenario                                                                  | Common Questions                                                                                                                                                                                                                       | National minimum requirement                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |
|---------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1. Movement between inpatient / residential, and ambulatory settings      | Do NOCC clinical and consumer ratings need to be recorded for the end of the ambulatory episode as well as the beginning of the inpatient / residential episode when a consumer is transferred from ambulatory care to bed-based care? | Discharge ratings for the clinician and consumer-rated measures are not required by the ambulatory service or the consumer respectively, when the reason for the closure of the ambulatory episode is transfer to psychiatric inpatient or community residential care.                                                                                                                                                                                                                                    |
| 2. Transfer between two wards of the psychiatric unit                     | Is the transfer of a patient from one psychiatric ward to another within the same hospital campus a new episode and thus requiring new data collection?                                                                                | No, because there has not been a change of treatment setting. However, there may be good clinical reasons to reassess the patient when transfer occurs eg, K10+ when the transfer is from an acute to a rehabilitation ward, or from a general acute unit to a forensic ward within the hospital. Decisions about whether such additional ratings are required need to be resolved at the local level. Where they do occur, they should be reported, and Reason for Collection coded as 'Review – Other'. |
| 3. Transfer between psychiatric units from one hospital campus to another | Should a new inpatient episode be commenced when a consumer is transferred from one hospital to another within the same mental health care organisation?                                                                               | Yes. Even though this is not technically a change in treatment setting, States and Territories have agreed that an inpatient episode should be recorded in these circumstances, with the associated data collection requirements.                                                                                                                                                                                                                                                                         |
| 4. Transfer of care between community teams                               | Does a new cycle of data collection begin when case management is transferred from one ambulatory care team to another within the same organisation?                                                                                   | No, within the national episode model the consumer is regarded as remaining within the same episode of care. However, as in the example (2) above, there may be good clinical reasons to reassess the patient when between-team transfer occurs. For example, transfer from crisis team to continuing care team. Decisions about whether such additional ratings are required need to be resolved at the local level.                                                                                     |



| Scenario                                        | Common Questions                                                                                                  | National minimum requirement                                                                                                                                                                                                                                                                                                                                                                                                                          |
|-------------------------------------------------|-------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 5. Multiple team involvement in case management | Is each team expected to complete ratings on the consumer?                                                        | No, the consumer is regarded as receiving only one episode of care at a time. Decisions about which team (or clinician) is responsible for completing the required ratings need to be at the local level. In general, this is expected to be the service unit that is principally responsible for providing treatment and care during the current Episode of Mental Health Care.                                                                      |
| 6. 'Intended' same day admissions               | Is each day of care a new inpatient episode, requiring a full set of ratings?                                     | No. Definitions developed under the National Survey of Mental Health Services since 1994, and now replicated in NMDS – Mental Health Establishments, regard 'intended same day admissions' as a component of ambulatory care services.                                                                                                                                                                                                                |
| 7. Discharge from hospital on indefinite leave  | Does an inpatient episode continue when a patient is placed on extended leave but remains, legally, an inpatient? | This is a common but complex issue in mental health services. As a general rule, it is recommended that, for the purposes of the NOCC dataset, the inpatient episode is deemed to have ended when the patient is sent on leave and where there is no intention that he/she return for an overnight stay within the next 7-day period.                                                                                                                 |
| 8. Return to hospital from indefinite leave     | Does a new inpatient episode begin when a patient returns to hospital after a period of extended leave?           | This is the converse of the above. It is recommended that where an inpatient episode is deemed to have ended as a result of indefinite leave, and the patient returns unexpectedly, a new inpatient episode should be commenced.                                                                                                                                                                                                                      |
| 9. Brief inpatient episodes                     | Are discharge ratings required for very brief inpatient episodes?                                                 | In general, yes, but there are exceptions:<br><br>For inpatient episodes in all Age Groups where the episode is of 3 days or less duration: the HoNOS/HoNOS65+/HoNOSCA is not required.<br><br>For all Child and Adolescent episodes of less than 21 days duration, the discharge SDQ is not required.<br><br>In both instances above, the exclusion is because the period that would be rated at discharge would overlap with the admission ratings. |

| Scenario                                                               | Common Questions                                                                                                                                                            | National minimum requirement                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
|------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                                                        |                                                                                                                                                                             | Apart from the above exceptions, all other aspects of the collection protocol are required at discharge from inpatient episodes.                                                                                                                                                                                                                                                                                                                                                           |
| 10. Consumers seen regularly but at intervals of greater than 3 months | How should the 3 monthly (91 day) review 'rule' be applied in these cases? Does it mean that they will need to be seen more regularly?                                      | No, definitely not, the collection protocol is intended to support good practice rather than dictate how services should be delivered. Where the needs of a consumer require that they be seen regularly but at greater than 3 monthly intervals, then reviews using the standard instruments should be conducted on the next appointment that occurs after 3 months have elapsed since the last collection occasion.                                                                      |
| 11. Admission to general medical (non- mental health) ward             | Is a new episode of mental health inpatient care commenced when the person is admitted to a (non-mental health) medical ward for the primary purpose of mental health care? | No. This is a continuation of the ambulatory episode. It is recommended however that a review of the consumer be conducted at this stage.                                                                                                                                                                                                                                                                                                                                                  |
| 12. Consultation Liaison teams                                         | What is expected of C-L teams in terms of collection of the NOCC data?                                                                                                      | <p>Consultation liaison is explicitly included as in scope for collection when there is a face-to- face assessment, with clarification that episodes extending over 14 days or less do not require discharge outcome measures to be collected.</p> <p>Outcome measures are to be collected at admission for all face-face assessments and are to be collected at discharge when the episode of care (defined by the elapsed days from admission to discharge) is greater than 14 days.</p> |



## Appendix 2: Fig. 9.1 Data to be reported at each Collection Occasion within each Episode Service Setting, for consumers in each Age Group

| Adult Inpatient         |                                                                              |                                    |
|-------------------------|------------------------------------------------------------------------------|------------------------------------|
| Collection Occasion     | AMHCC Requirement                                                            | PSOLIS NOCC Requirement            |
| Admission Collection    | MHPoC, HoNOS, Diagnosis                                                      | K10+, HoNOS, Diagnosis             |
| Review Collection       | MHPoC, HoNOS, Diagnosis – measures only required if MHPoC changes            | K10, HoNOS, Diagnosis              |
| Discharge Collection    | Nil                                                                          | K10, HoNOS, Diagnosis              |
| Adult Ambulatory        |                                                                              |                                    |
| Collection Occasion     | AMHCC Requirement                                                            | PSOLIS NOCC Requirement            |
| Activation Collection   | MHPoC, HoNOS, LSP-16, Diagnosis                                              | K10+, HoNOS, Diagnosis             |
| Review Collection       | MHPoC, HoNOS, LSP-16, Diagnosis – measures only required if MHPoC changes    | K10, HoNOS, LSP-16, Diagnosis      |
| Deactivation Collection | Nil                                                                          | K10, HoNOS, LSP-16, Diagnosis      |
| Older Adult Inpatient   |                                                                              |                                    |
| Collection Occasion     | AMHCC Requirement                                                            | PSOLIS NOCC Requirement            |
| Admission Collection    | MHPoC, HoNOS65+, RUG-ADL, Diagnosis                                          | K10+, HoNOS 65+, RUG-ADL,Diagnosis |
| Review Collection       | MHPoC, HoNOS65+, Diagnosis – measures only required if MHPoC changes         | K10, HoNOS 65+, RUG-ADL, Diagnosis |
| Discharge Collection    | Nil                                                                          | K10, HoNOS 65+, Diagnosis          |
| Older Adult Ambulatory  |                                                                              |                                    |
| Collection Occasion     | AMHCC Requirement                                                            | PSOLIS NOCC Requirement            |
| Activation Collection   | MHPoC, HoNOS65+, LSP-16, Diagnosis                                           | K10+, HoNOS 65+, Diagnosis         |
| Review Collection       | MHPoC, HoNOS65+, LSP-16, Diagnosis – measures only required if MHPoC changes | K10, HoNOS65+, LSP-16, Diagnosis   |
| Deactivation Collection | Nil                                                                          | K10, HoNOS65+, LSP-16, Diagnosis   |

Source: AMHCC PSOLIS Business Operating Rules v 1.0 .31 Oct 2016 and amhocn.org

## Notes to Appendix 2

- This table identifies the national reporting requirements and is not intended to restrict a State or Territory from the collection of additional data at specific collection occasions.
- Discharge ratings for the clinician and consumer-rated measures (i.e., the HoNOS/CA/65+, LSP-16, FIHS, the SDQ, BASIS-32/K10/MHI-38) are not required by the ambulatory service or the consumer respectively, when the reason for the closure of the ambulatory episode is transfer to a bed-based treatment service setting of that organisation (i.e., psychiatric inpatient or community residential service). Where possible, the clinician and consumer measure ratings for the admission NOCC in the inpatient unit or the community residential care setting are reported as the consumer's discharge ratings from the ambulatory episode. Details are required, however, regarding Principal and Additional Diagnoses and Mental Health Legal Status relevant to the ambulatory episode of care.
- Discharge ratings for the clinician and consumer-rated measures (i.e., the HoNOS/CA/65+, LSP-16, FIHS, the SDQ, BASIS-32/K10/MHI-38) are not required for brief ambulatory episodes. Brief ambulatory episodes are those where the number of days between admission to and discharge from the episode of care is 14 days or less duration. Details are required, however, regarding Principal and Additional Diagnoses and Mental Health Legal Status relevant to the ambulatory episode of care.
- Discharge ratings for the HoNOS, HoNOS65+ and HoNOSCA are not required for inpatient episodes of 3 days or less duration.
- Discharge ratings for the SDQ are not required for any episode of less than 21 days duration because the rating period used at discharge (previous month) would overlap significantly with the period rated at admission.
- The classification of consumer rated measures as mandatory is intended only to indicate the expectation that consumers will be offered to complete self-report measures at the specified Collection Occasions. There are circumstances where offering such measures will not be appropriate and special considerations applying to the collection of consumer rated measures are described in Special considerations applying to the collection of consumer self-report and parent measures.
- The K10L3D is a variation of the K10 designed for use in inpatient settings where the episode is of less than 3 days duration.

## **Appendix 3: Process for Community (Adult and Older Adult)**

The information collected in the NOCC may assist in the creation or revision of the treatment plan by identifying areas where significant problems exist and guide future clinical care.

### **Activation**

The Care Coordinator collects the NOCC data at point of activation and enters it into PSOLIS. A printed copy of the NOCC results should be placed in the health record. NOCC data may include, but not be limited to, HoNOS/HoNOS65+, LSP-16 and Kessler10/10+.

### **Review**

The NOCC review should occur within 3 months (91 days) of the activation of the consumer. The Review NOCC will be collected and entered into PSOLIS by the Care Coordinator and a printed copy of the NOCC results should be placed in the health record. NOCC data may include, but not be limited to, HoNOS/HoNOS65+, LSP-16 and Kessler10/10+.

### **Deactivation**

At deactivation the Care Coordinator should complete the NOCC data and enter the data into PSOLIS and a printed copy of the NOCC results should be placed in the health record. NOCC data may include, but not be limited to, HoNOS/HoNOS65+, LSP-16 and Kessler10/10+.

This includes a change in treatment setting such as when a community consumer requires admission to an inpatient unit. A printed copy of the NOCC results should be placed on the health record.

## **Appendix 4: Process for Inpatient (Banksia, Karri, Moodjar and Yorgum inpatient units)**

Care should always be taken to ensure the correct NOCC measures are recorded according to the age of the patient, not the ward in which they are admitted.

### **Admission**

The admitting nurse collects the NOCC data on admission and enters it into PSOLIS. A printed copy of the NOCC results should be placed in the health record. NOCC data may include, but not be limited to, HoNOS/HoNOS65+ and Kessler10/10+.

### **Review**

The NOCC review should occur within three months (91 days) of the admission of the consumer. The Review NOCC will be collected by the allocated nurse and entered in PSOLIS. A printed copy of the NOCC results should be placed in the health record. NOCC data may include, but not be limited to, HoNOS/HoNOS65+ and Kessler10/10+.

### **Discharge**

The discharge nurse should complete the discharge NOCC and enter it in PSOLIS. A printed copy of the NOCC results should be placed in the health record. NOCC data may include, but not be limited to, HoNOS/HoNOS65+ and Kessler10/10+.

### **Resource Utilisation Groups – Activities of Daily Living (RUG-ADL)**

This assessment is to be completed in conjunction with the NOCC data at admission and review in the Banksia inpatient unit only for patients over the age of 65.